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Question 1

Not answered

Marked out of 4.00

Neuromodulation Therapies

For each of the clinical and treatment scenarios given below, match the most appropriate and likely intervention.

A 76-year old man is experiencing a severe depressive episode. He is mute and has not taken in food or fluids for two days. The treatment he receives causes some muscular aches, some anterograde and retrograde amnesia

A 47-year old woman has very severe OCD. Her condition has not improved with medication and psychotherapy treatment. The treatment she is offered involves the use of gamma radiation and may adversely affect her personality

A 50-year old woman has recurrent moderate depressive episodes that have not been successfully treated with several trials of medication. The treatment she receives targets the dorsolateral prefrontal cortex and does not produce amnesia as a side-effect

A 24-year old man has severe tics associated with Tourette's syndrome, which significantly impair his functioning. The treatment he receives involves the placement of a subdermal pacemaker to reduce the frequency of his tics

Your answer is incorrect.

Explanation:

- 1) Some common side-effects of ECT include antero- and retrograde amnesia (usually not long-lasting), headache and muscle aches.
- 2) The treatment described is psychosurgery, which may employ gamma radiation to target limbic structures. One less common yet important potential side-effect is an alteration of the personality.
- 3) Transcranial Magnetic Stimulation can be used in the treatment of depressive illness. Unlike ECT, it does not produce amnesia as a side-effect.
- 4) Deep brain stimulation can be used to reduce the motor tics associated with Tourette's. A subdermal pacemaker is inserted which produces electrical impulses targeting areas of the brain related to motor tics.

The correct answer is: A 76-year old man is experiencing a severe depressive episode. He is mute and has not taken in food or fluids for two days. The treatment he receives causes some muscular aches, some anterograde and retrograde amnesia → Electroconvulsive Therapy, A 47-year old woman has very severe OCD. Her condition has not improved with medication and psychotherapy treatment. The treatment she is offered involves the use of gamma radiation and may adversely affect her personality → Psychosurgery, A 50-year old woman has recurrent moderate depressive episodes that have not been successfully treated with several trials of medication. The treatment she receives targets the dorsolateral prefrontal cortex and does not produce amnesia as a side-effect → Transcranial Magnetic Stimulation, A 24-year old man has severe tics associated with Tourette's syndrome, which significantly impair his functioning. The treatment he receives involves the placement of a subdermal pacemaker to reduce the frequency of his tics → Deep Brain Stimulation



Question 2

Not answered

Marked out of 5.00

Human Rights in Mental Health

The following scenarios describe contraventions of an article of the Human Rights Act. Match each scenario with the article it contravenes.

A young man is admitted to the ward with first episode psychosis. His family presents to the ward wishing to visit him, and they are refused entry. They are declined access to visit their relative during his entire admission to hospital

A woman is detained under the Mental Health Act (1983) for an episode of acute mania. She is an inpatient for four months, and at no point during this time is a mental health tribunal organised

A man is admitted to the ward for assessment and treatment of a severe depressive illness. He is Jewish, and his family express a wish, on his behalf, that he be provided with Kosher food during his stay in the hospital. This request is not adhered to, and the young man is often served pork

A young man with a psychotic illness is placed in seclusion. He has an aggressive outburst during which staff intervene to restrain him. During this episode his clothes are torn by the staff, he loses control of his bowels, he sustains bruises to his arms, and he is left in an undressed and unwashed state for an hour after the restraint

A young woman with suicidal ideation is admitted to the ward and not observed or made contact with for two hours after admission. She is later discovered to have hanged herself

Your answer is incorrect.

Explanation:

The right to private/family life includes issues arising about family visits, home leaves, restricting correspondences, and prohibiting activities such as smoking in care homes.

The right to a fair hearing cannot be interfered with under any circumstance. Applications must be made to mental health tribunals and Court of Protection.

The prohibition of torture comprises any act that is inhuman or degrading. These issues are most likely to arise with seclusion and detentions.

The state has a duty to protect lives. All deaths occurring in state detention must be investigated, including suicides. It applies to both formal and informal patients.

The correct answer is: A young man is admitted to the ward with first episode psychosis. His family presents to the ward wishing to visit him, and they are refused entry. They are declined access to visit their relative during his entire admission to hospital → Article 8: Right to private/family life, A woman is detained under the Mental Health Act (1983) for an episode of acute mania. She is an inpatient for four months, and at no point during this time is a mental health tribunal organised → Article 6: Right to a fair hearing, A man is admitted to the ward for assessment and treatment of a severe depressive illness. He is Jewish, and his family express a wish, on his behalf, that he be provided with Kosher food during his stay in the hospital. This request is not adhered to, and the young man is often served pork → Article 9: Freedom of thought and religion, A young man with a psychotic illness is placed in seclusion. He has an aggressive outburst during which staff intervene to restrain him. During this episode his clothes are torn by the staff, he loses control of his bowels, he sustains bruises to his arms, and he is left in an undressed and unwashed state for an hour after the restraint → Article 3: Prohibition of torture, A young woman with suicidal ideation is admitted to the ward and not observed or made contact with for two hours after admission. She is later discovered to have hanged herself → Article 2: Right to life



Question 3

Not answered

Marked out of 4.00

Distribution of mental disorders

Select the most appropriate numerical data for each of the following epidemiological observations in the UK

Number of adults experiencing at least one mental health problem in their lifetime

Number of patients with a depressive episode that develop a chronic course of the illness.

Number of persons aged >65 living in the community that have depression at any given time

Number of people living with a mental health problem at any given time in the UK.

Your answer is incorrect.

Explanation:

1) According to the Adult Psychiatric Morbidity in England, 2007, One in four will experience an episode of mental illness at some point in their lifetime.

2) The WHO study of mental disorders was conducted in 14 centres across the world. They found that 50% of patients still had a diagnosis of depression 1 year later (Simon et al., 2002) and at least 10% had persistent or chronic depression (Kessler et al., 2003).

3) A systematic review by Beekman et al (1999) concluded that Major depression, diagnosed according to rigorous research diagnostic criteria is relatively rare among the elderly (1.8% prevalence). However, minor depression was more common at a rate of 10.2%. Considering all depressive syndromes deemed clinically relevant, the average prevalence was 13.3% (nearly 1 in 7).

4) According to the Adult Psychiatric Morbidity in England, 2007, at any one time, at any one time, one in six people will be living with a mental health problem

The correct answer is: Number of adults experiencing at least one mental health problem in their lifetime → One in four, Number of patients with a depressive episode that develop a chronic course of the illness. → One in ten, Number of persons aged >65 living in the community that have depression at any given time → One in seven, Number of people living with a mental health problem at any given time in the UK. → One in six



Question 4

Not answered

Marked out of 4.00

Prevention of Psychosis

Identify one correct option for each of the following preventive measures in schizophrenia

Targeting the urban social milieu to reduce incidence of schizophrenia

Choose...

Introducing schemes to identify a preschizophrenic state

Choose...

Targeting pregnant women addressing low birth weight to diminish the burden of schizophrenia

Choose...

Setting up help groups for young cannabis smokers to reduce psychosis

Choose...

Your answer is incorrect.

Explanation: **Universal prevention**/interventions are directed to a general population group. Some epidemiological factors for schizophrenia such as urban living, poor social capital including low social cohesion, exclusion and discrimination, low socioeconomic status are so widespread that their targeting can be considered a universal intervention. Increasing social capital, decreasing discrimination and reducing poverty are possible goals of universal intervention. **Indicated interventions** target high-risk individuals with minimal but detectable signs or symptoms foreshadowing mental disorder, but who do not meet diagnostic levels. E.g Identification of a preschizophrenic state/prodrome of schizophrenia. **Selective interventions** are targeted to individuals or subgroups whose risk of developing disorder is significantly higher than average. Perinatal complications and cannabis use have stimulated interest as putative risk factors for schizophrenia. Perinatal complications such as low birth weight, hypoxic events during delivery, small for gestational age and premature rupture of membranes. Cannabis has been strongly debated in the etiology of schizophrenia. The pooled estimate for development of psychosis associated with prior cannabis use is an odds ratio of 2.1.

The correct answer is: Targeting the urban social milieu to reduce incidence of schizophrenia → Universal prevention, Introducing schemes to identify a preschizophrenic state → Indicated prevention, Targeting pregnant women addressing low birth weight to diminish the burden of schizophrenia → Selective prevention, Setting up help groups for young cannabis smokers to reduce psychosis → Selective prevention



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